hovid

hovid-Salbutamol Syrup 2mg / 5mL (SUGAR FREE)

VISAL02-PC (MY)

DESCRIPTION
Strawberry flavoured, colourless and transparent liquid.

COMPOSITION
Salbutamol sulphate 2.41 mg equivalent to salbutamol 2mg / 5mL

PHARMACODYNAMICS
Salbutamol is a selective ß2 adrenoceptor agonist. At therapeutic doses it acts on the ß2 adrenoceptors of bronchial muscle, with little or no action on the ß1 adrenoceptors of cardiac muscle.

PHARMACOKINETICS
Salbutamol administered intravenously has a half-life of 4 to 6 hours and is cleared partly renally and partly by metabolism to the inactive 4'-O-sulphate (phenolic sulphate) which is also excreted primarily in the urine. The faeces are a minor route of excretion. The majority of a dose of salbutamol given intravenously, orally or by inhalation is excreted within 72 hours. Salbutamol is bound to plasma proteins to the extent of 10%.
After oral administration, salbutamol is absorbed from the gastrointestinal tract and undergoes considerable first-pass metabolism to the phenolic sulphate. Both unchanged drug and conjugate are excreted primarily in the urine. The bioavailability of orally administered salbutamol is about 50%.

INDICATION
Relief of bronchial asthma of all types, chronic bronchitis and emphysema.

CONTRAINDICATION
Salbutamol preparations are contraindicated in patients with a history of hypersensitivity to any of their components.

WARNINGS AND PRECAUTIONS
The management of asthma should normally follow a stepwise programme, and patient response should be monitored clinically and by lung function tests.

Increasing use of short-acting inhaled ß2 agonists to control symptoms indicates deterioration of asthma control. Under these conditions, the patient"s therapy plan should be reassessed. Sudden and progressive deterioration in asthma control is potentially life-threatening and consideration should be given to starting or increasing corticosteroid therapy. In patients considered at risk, daily peak flow monitoring may be instituted.

Patients should be warned that if either the usual relief is diminished or the usual duration of action reduced, they should not increase the dose or its frequency of administration, but should seek medical advice.

Salbutamol should be administered cautiously to patients with thyrotoxicosis.

Potentially serious hypokalaemia may result from ß2 agonist therapy mainly from parenteral and nebulised administration. Particular caution is advised in acute severe asthma as this effect may be potentiated by concomitant treatment with xanthine derivatives, steroids, diuretics and by hypoxia. It is recommended that serum potassium levels are monitored in such situations.

In common with other ß-adrenoceptor agonists, salbutamol can induce reversible metabolic changes, for example increased blood sugar levels. The diabetic patient may be unable to compensate for this and the development of ketacidosis has been reported. Concurrent administration of corticosteroids can exaggerate this effect.

Tocolysis: Serious adverse reactions including death have been reported of salbutamol to women in labor. In the mother, these include increased heart rate, transient hyperglycaemia, hypokalaemia, cardiac arrhythmias, pulmonary oedema and myocardial ischaemia. Increased fetal heart rate and neonatal hypoglycaemia may occur as a result of maternal administration.

As maternal pulmonary oedema and myocardial ischaemia have been reported during or following treatment of premature labour with ß2 agonists, careful attention should be given to fluid balance and cardio-respiratory function, including ECG should be monitored. If signs of pulmonary oedema or myocardial ischaemia develop, discontinuation of treatment should be considered.

Bronchodilators should not be the only or main treatment in patients with severe or unstable asthma. Severe asthma requires regular medical assessment including lung function testing as patients are at risk of severe attacks and even death. Physicians should consider using oral corticosteroid therapy and/or the maximum recommended dose of inhaled corticosteroid in those patients.

Effect on ability to drive and use machines - none known

PREGNANCY AND LACTATION
Administration of drugs during pregnancy should only be considered if the expected benefit to the mother is greater than any possible risk to the foetus. As with the majority of drugs, there is little published evidence of its safety in the early stages of human pregnancy, but in animal studies there was evidence of some harmful effects on the foetus at very high dose levels.

As salbutamol is probably secreted in breast milk, its use in nursing mothers is not recommended unless the expected benefits outweigh any potential risk. It is not known whether salbutamol in breast milk has a harmful effect on the neonate.

DRUG INTERACTIONS
Salbutamol and non-selective beta-blocking drugs,such as propranolol, should not usually be prescribed together.

MAIN SIDE / ADVERSE EFFECTS
- **Immune system disorders**
  Hypersensitivity reactions including angioedema, urticaria, bronchospasm, hypotension and collapse
- **Metabolism and nutrition disorders**
  Hypokalaemia.
- **Nervous system disorders**
  Tremor, headache, hyperactivity
- **Cardiac disorders**
  Tachycardia, palpitations, cardiac arrhythmias including atrial fibrillation, supraventricular tachycardia and extrasystoles.
- **Vascular disorders**
  Peripheral vasodilatation.
- **Musculoskeletal and connective tissue disorders**
  Muscle cramps, feeling of muscle tension.

OVERDOSE AND TREATMENT
The most common signs and symptoms of overdose with salbutamol are transient beta agonist pharmacologically mediated events, including tachycardia, tremor, hyperactivity and metabolic effects including hypokalaemia and lactic acidosis. Hypokalaemia may occur following overdose with salbutamol. Serum potassium levels should be monitored. Nausea, vomiting and hyperglycaemia have been reported, predominantly in children and when salbutamol overdose has been taken via the oral route.

Treatment consists of discontinuation of salbutamol together with appropriate symptomatic therapy. Administer a cardioselective ß-adrenergic blocker (e.g.acebutalol, atenolol, metoprolol), if necessary for cardiac arrhythmias. However, ß-adrenergic blocker should be used with caution because it could induce severe bronchospasm.

DOSAGE AND ADMINISTRATION
Adults
Oral, 2 to 4 mg three or four times daily, which may be increased to a maximum of 8 mg (20 mL syrup) three or four times a day if adequate bronchodilation is not obtained. The minimum starting dose is 2 mg three times a day given as 5 mL syrup.

Children
- *2 to 6 years:* The minimum starting dose is 1mg as 2.5 mL of syrup three times daily. This may be increased to 2 mg as 5 mL of syrup three or four times daily.
- *6 to 12 years:* The minimum starting dose is 2 mg as 5 mL syrup three times daily. This may be increased to four times daily.
- *Over 12 years:* The minimum starting dose is 2 mg three times daily given as 5 mL syrup. This may be increased to 4 mg as 10 mL syrup three or four times daily.

hovid-Salbutamol Syrup 2mg/5mL (sugar free) is well tolerated by children so that, if necessary, these doses may be cautiously increased to the maximum dose. For lower doses the syrup may be diluted with freshly prepared purified water BP.

Elderly
In elderly patients or in those known to be unusually sensitive to beta-adrenergic stimulant drugs, it is advisable to initiate treatment with the minimum starting dose.

Note: The information given here is limited. For further information, consult your doctor or pharmacist.

Storage: Store below 30˚C.

Presentation / Packing:
Syrup 2 mg / 5 mL (Sugar Free) x 60 mL, 100 mL

Product Registration Holder /
Manufactured by: HOVID Bhd.
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30010 Ipoh, Malaysia.

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